FMD | FINDMYDENTIST

Master Product Requirements Document

Unified dentistry-focused digital ecosystem · Product, workflows and engineering requirements

Purpose
A developer-facing master reference defining FMD's product vision, apps, roles, features, user journeys, AI capabilities, architecture expectations, safeguards, commercial rules and acceptance criteria.

Audience: FMD founders, product owners, designers, developers, QA, operations and integration partners.

Baseline: 2 October 2026 · Working requirements document

This document describes the complete intended product vision. It does not divide functionality into V1 and later phases. The development team should separately identify dependencies, estimate scope, and document what can be delivered under the agreed commercial budget.

Document map

1. Product overview and principles
2. Ecosystem and product boundaries
3. User types, roles and permissions
4. FMD Main public app
5. FMD Concierge enquiry service
6. FMD Clinic OS
7. FMD Clinic agentic AI manager
8. FMD Scholar standalone app
9. FMD Admin and Accountant Portal
10. Shared platform capabilities
11. Technical architecture and integrations
12. Core data entities
13. Security, privacy, clinical safety and compliance
14. Commercial model and entitlements
15. Notifications and communications
16. Analytics and reporting
17. Non-functional requirements
18. End-to-end user journeys
19. Acceptance criteria and QA
20. Delivery and handover
21. Open decisions and assumptions

1. Product overview and principles

Product definition. FindMyDentist (FMD) is a dentistry-only ecosystem connecting patients, dentists, clinics, dental students and dental companies through AI-led guidance, discovery, content, clinic operations, academic tools and care coordination.

Vision

Make dental care easier to discover and navigate, help dental practices operate with less administrative burden, and give dental learners and professionals specialized AI capabilities.

Product principles

- Stay focused on dentistry and dental-care workflows rather than becoming a general healthcare marketplace.
- Build useful outcomes into the experience: finding information, advancing a workflow, completing an authorized task, or maintaining continuity.
- Use a consistent FMD brand across products while allowing each app to serve a distinct user need.
- Keep AI model-agnostic where practical so providers can be changed without rebuilding core workflows.
- Protect patient and clinical information through consent, least-privilege access and auditability.
- Clearly distinguish educational information, AI-generated drafts, recommendations and verified professional decisions.
- Do not represent AI as a substitute for a registered dental professional.
- Make subscriptions, entitlements, limits and sponsored placements transparent.

Value propositions

Audience	Value
Patients	Understand dental topics, discover providers, access content, book or coordinate care, and organize dental records.
Dentists and clinics	Operate the practice and use an agentic AI assistant to execute authorized administrative and workflow tasks.
Dental students	Use a dentistry-focused academic AI workspace for doubts, viva, presentations, research and study workflows.
Dental companies	Maintain a corporate presence and participate in relevant dental ecosystem opportunities.
FMD internal team	Operate platform services, manage trust and safety, and respond to Concierge enquiries.
Accountant	Manage company accounting, filings workflows, reconciliations and reporting through a dedicated portal.

2. Ecosystem and product boundaries

Product	Definition	Boundary
FMD Main	Public app for patients, dentists, students and dental companies. Includes AI companion, discovery, content, social features, booking, records and Concierge enquiries.	Public-facing app
FMD Clinic OS	Clinic management software centered on an agentic AI personal clinic manager.	Separate clinic product
FMD Scholar	Standalone dental academic and research AI app. Student access is tied to FMD Main student membership.	Separate app
FMD Admin	Internal web administration system for platform operations and business oversight.	Internal system
Accountant Portal	Finance workspace within FMD Admin for the accountant's independent day-to-day work.	Restricted finance role/interface
FMD Concierge	Patient-submitted enquiry/request. FMD team personally follows up and coordinates care.	Service within FMD Main; not a separate app
FMD Campus	Institutional campus app.	Deleted; exclude from current product

No V1-versus-later-feature split is used in this product definition. Do not add FMD Campus or a standalone Concierge app to navigation, architecture, estimates or deliverables.

Account and entitlement model

- Use a shared identity layer where practical, with app-specific roles, permissions and entitlements.
- FMD Main student membership grants access to FMD Scholar under the included usage allowance.
- FMD Main professional membership and FMD Clinic OS subscription are separate products and charges.
- Support clinic organizations with multiple staff members and role-specific access.
- Provide account linking, recovery, consent, and deletion/request workflows.

3. User types, roles and permissions

Role	Core capabilities	Controls
Patient	AI companion, discovery, appointments, content, own records and Concierge enquiries.	Own data only; explicit sharing for records.
Dentist	Professional profile, content, bookings and authorized clinic workflows.	Verification and organization-level permissions.
Student	FMD Main student membership and FMD Scholar access.	Eligibility checks where required.
Dental company/corporate	Corporate account and permitted company profile/promotional features.	Company verification and content/advertising rules.
Clinic owner/admin	Clinic settings, team, schedule, operational and financial configuration.	Strong authentication for sensitive actions.
Clinic staff	Assigned patient, appointment and operations workflows.	Least privilege; restrict clinical and finance access by role.
Clinic AI manager	Uses tools enabled for the clinic and requesting user.	Tool scopes, consent, approval policies, logs and rate limits.
FMD operations/support	Approved service and support workflows.	Scoped, logged access.
Moderator	Content review and reports.	No unnecessary access to clinical records.
FMD super admin	Platform configuration, role management and oversight.	Privileged actions logged and protected.
Accountant	Accounting records, reconciliations, tax-support data, filing workflows and finance reports.	No default access to clinical or unrelated user data.

Permissions must be scoped by organization, clinic, record, action and data sensitivity where applicable. Avoid broad access based solely on a role label.

4. FMD Main public app

FMD Main is AI-first. The branded assistant is the primary entry point and guides users into the app's discovery, content, booking, records and service features. It does not independently execute patient tasks.

4.1 Onboarding and account

- Registration and secure authentication using phone/email verification or equivalent.
- Account type selection: patient, dentist, student or dental company/corporate.
- Collect only necessary profile details and explain sensitive-data use.
- Terms, privacy notices, consent, communication preferences, recovery and account deletion request flows.
- Membership screen showing status, benefits, renewal date, usage allowance and applicable limits.
- Profile editing and account linking without cross-user data exposure.

4.2 AI-first home and assistant

- Prominent AI companion entry point with text, voice and supported image/document inputs.
- Unified branded assistant adapts responses and capabilities to the user's role while remaining one assistant experience.
- Support English, Telugu, Kannada, Tamil, Hindi and additional languages as supported by selected speech/model services. Allow language switching and mixed-language interaction where technically supported.
- Explain dental concepts in plain language and provide educational next steps.
- Use uncertainty-aware wording; do not make definitive diagnoses or prescribe treatment.
- Provide appropriate escalation guidance for concerning symptoms using clinically reviewed content.
- Let users revisit conversations subject to privacy and retention controls.

- AI may guide a patient through finding a provider or booking, but must present choices and require the patient to complete/confirm actions. No autonomous booking, contacting clinics or other task execution by public AI.

4.3 AI guidance and broader dental companion functions

- Help users organize appointment information, treatment plans and reminders.
- Help create a dental treatment budget worksheet based on user-entered or provider-supplied estimates; clearly label estimates and exclusions.
- Explain dental insurance documents and help users organize claim-related information without guaranteeing coverage or claim approval.
- Summarize uploaded reports in understandable language, with limitations and recommendation to consult the treating dentist.
- Help organize dental history, preferences and prior conversations with explicit consent and controls.
- Offer checklists and reminders related to dental care. Notifications should be user-controlled and not imply clinical monitoring.

4.4 Dentist and clinic discovery

- Search by location, specialty, service, availability, language and supported filters.
- Show provider profiles with qualifications, experience, services, clinic details, hours, photos and contact/booking options.
- Clearly distinguish verification from paid promotion; label sponsored and featured placements.
- Support claim-profile and provider verification workflows.
- Allow AI to recommend relevant providers based on user-stated concern, location and preferences. Explain that matching is informational and not a diagnosis.
- Display current availability only when connected to a reliable scheduling source; otherwise label availability as needing confirmation.

4.5 Appointment discovery and booking

- Browse provider availability, appointment types and relevant booking information.
- Collect appointment preferences and display the selected provider, time, clinic, fee information and cancellation terms before user confirmation.
- Require explicit patient confirmation before creating a booking or submitting a request.
- Show booking status, reminders, cancellation/reschedule routes and support contact.
- Handle unavailable slots, duplicate requests, failed payments and provider-side changes gracefully.
- Support clinic-controlled booking rules and appointment buffers where applicable.

4.6 Digital dental records

- Patient-controlled storage for reports, X-rays, photos, treatment plans, prescriptions and other supported dental documents.
- Organize by date, clinic, provider, treatment episode and document type.
- Allow user download, export, deletion requests and explicit sharing with a selected provider.
- Show upload progress, file validation, storage limits and access history where appropriate.
- Use secure object storage and access-controlled, expiring links; never expose raw storage paths publicly.

4.7 Explore content and social experience

- Content feed includes dental reels/short videos, posts, educational material, polls and relevant creator content as supported.
- Personalize feed using searches, declared interests, dental concerns and relevant care journey context with consent and controls.
- Show search-related content prominently when it directly addresses the user's query or concern.
- Include general dental education and discovery content so the feed is not restricted to prior searches.
- Support creator/dentist profiles, follows, likes, comments, saves, reports and moderation rules as specified for the chosen release scope.

- Clearly label sponsored content and paid visibility.
- Allow authorized dental professionals to create posts and upload media subject to platform rules and patient consent requirements.
- Do not publish identifiable patient case material without documented, valid consent and any required review.

4.8 Dentist profiles and professional presence

- Profile photo, professional description, verified qualifications, specialty/interest areas, clinic affiliations and contact/booking details.
- Content portfolio and engagement features subject to moderation.
- Membership status and profile editing controls.
- Claims must not be misleading; verification status must reflect actual checks performed.

4.9 Jobs and professional opportunities

- Create, browse and search dental jobs or opportunities.
- Company/clinic posting permissions, listing details, location, role, requirements and application route.
- Report, moderation, expiry and renewal workflows.
- Do not imply that FMD verifies every employer or job claim unless a specific verification process has been completed.

4.10 Public AI interaction boundary

FMD Main AI advises and navigates; FMD Clinic AI executes authorized work. FMD Main may help users prepare an action, show options and direct them to the relevant workflow. It must not autonomously book appointments, message providers, submit claims, make payments or take other consequential actions on a patient's behalf.

5. FMD Concierge enquiry service

Concierge is a premium-care enquiry and human coordination service embedded in FMD Main. A patient submits a request; the FMD team contacts the patient and coordinates next steps personally.

Patient-facing flow

- User opens Concierge section and reviews service description, geographic availability, expected response process and any applicable terms.
- User submits enquiry with contact preference, location/area, dental-care need, preferred timing and optional notes.
- Collect only necessary health information. Explain how submitted information will be used and who may contact the patient.
- Show enquiry reference and confirmation that the request was received—not a promise that a service or appointment is confirmed.
- FMD team follows up personally, clarifies needs, and coordinates with participating clinics or providers as appropriate.
- Any clinic selection, transport, second opinion, treatment plan, fee or service commitment is confirmed with the patient before it is finalized.

Internal handling

- Enquiry queue with status, priority, assigned coordinator, timestamps and next action.
- Statuses such as New, Contact Attempted, Contacted, Needs Clarification, Coordinating, Awaiting Patient, Confirmed, Closed and Cancelled.
- Secure notes, contact history, consent records and service checklist.
- Escalation and reassignment when an enquiry is overdue.
- Operational reports for response times, conversion, cancellations and service quality.
- Do not represent Concierge as a 24/7 staffed service unless staffing and response commitments are established.

Concierge is not a separate application and does not imply that FMD itself provides clinical treatment. Treatment is delivered by appropriately qualified providers.

6. FMD Clinic OS

FMD Clinic OS is a clinic-management platform whose primary commercial differentiator is an agentic AI personal clinic manager that can perform permitted work. The core clinic system must remain usable even if AI services are unavailable.

6.1 Clinic setup and workspace

- Clinic registration, verification, profile, location, operating hours, services, appointment rules and billing configuration.
- Multiple branches/workspaces where supported, with clear separation of records and reporting.
- Invite staff, assign roles, configure access and deactivate accounts.
- Configure clinic communication channels, templates, language and escalation rules.
- Set AI permissions, allowed tools, approval thresholds and operating hours.

6.2 Dashboard and daily operations

- Today/next appointments, check-in status, cancellations, no-shows and pending confirmations.
- Patient follow-up queue, incomplete documentation alerts and operational tasks.
- Collections and outstanding balances based on recorded transactions.
- Inventory alerts, staff tasks and clinic performance summaries.
- Customizable widgets and date/branch filters.

6.3 Patient and clinical workflow management

- Patient registration, demographics, contact details, consent status and duplicate detection.
- Patient timeline with appointments, documents, treatment plans, communications and billing events.
- Clinical notes and structured fields configurable by clinic and discipline.
- Odontogram/charting where included in the agreed implementation; support tooth/surface notation and history without overwriting prior entries.
- Treatment plan and estimate preparation with itemized procedures, quantities, fees, discounts and patient acknowledgement.
- Attach X-rays, photographs, scans and reports with access controls.
- Record author, timestamps, edits and amendment history.
- Clinical records should be attributable to the responsible clinician; AI drafts require review and explicit acceptance.

6.4 Appointment and calendar management

- Day/week/month views, provider calendars, chair/room resources where configured, appointment types and duration.
- Book, reschedule, cancel, block time, manage waitlists and record attendance.
- Appointment status lifecycle with timestamps and actor attribution.
- Automated reminders and confirmation requests according to clinic policy.
- Conflict checks, duplicate prevention, buffers and working-hour rules.
- Patient communication must respect consent, channel rules and opt-out preferences.

6.5 Billing and payments

- Estimates, invoices, receipts, adjustments, discounts and payment records.
- Support multiple payment modes and partial payments as configured.
- Track outstanding balances, refunds and voids with permissions and audit history.
- Daily collection sheet and date-wise financial summaries.
- Separate clinic treatment revenue from FMD platform revenue.
- Integrate payment gateway only under an agreed settlement model; show who is merchant of record and where funds settle.

6.6 Inventory and procurement

- Item catalog, units, stock levels, reorder thresholds, supplier details and purchase records.
- Batch/expiry tracking where needed for relevant supplies.
- Stock movement history and adjustment reasons.
- Low-stock and expiry alerts.
- AI may draft a purchase order; sending or placing an order requires configured authorization.

6.7 Staff and task management

- Staff profiles, roles, assigned duties, task lists and completion status.
- Daily handover notes and operational checklists.
- Task assignment, due dates, priority and escalation.
- AI-generated task suggestions must identify their source and allow staff review.

6.8 Reports and clinic insights

- Appointment utilization, cancellation/no-show rates, recall completion and patient return metrics.
- Collections, outstanding balances, procedure mix and other configurable business summaries.
- Staff and branch summaries restricted by permissions.
- Export reports in common formats with date range and data-scope controls.
- Clearly define formulas and denominators; avoid presenting estimates as audited financial statements.

7. FMD Clinic agentic AI manager

Core USP: A branded, dentistry-focused AI bot that behaves like a personal clinic manager and executes individual tasks through authorized tools. It should feel like a task-capable assistant, not a generic chatbot. Underlying model/provider identity should not be the user-facing brand.

7.1 Interaction modes

- Chat interface with natural-language instructions and conversation context.
- Voice input/output where supported, including hands-free-friendly workflows with clear recording controls.
- Task cards showing request, plan, status, approvals, result and any follow-up needed.
- Scheduled and recurring tasks configured by clinic users.
- Manual run, review queue and history of prior tasks.
- Clear notification when an action is completed, partially completed, blocked or failed.

7.2 Task capability catalogue

Agent / task area	Examples of supported work	Execution controls
Appointments/front desk	Find unconfirmed appointments, draft confirmation messages, identify schedule gaps, prepare rescheduling options.	Outbound communication and booking changes follow clinic policy and approval settings.
Patient recall	Generate recall sheets, identify patients due for follow-up, prepare call/message lists and track outcomes.	Use configured clinical recall rules; avoid unsupported clinical recommendations.
Payments and accounts	Prepare daily payment sheets, summarize collections, identify missing entries and outstanding balances.	No autonomous bank transfer, refund or financial commitment unless separately authorized and safeguarded.
Content strategy	Suggest content calendar, topics, campaign ideas and audience-specific plans.	Clinic owner reviews strategy and claims.
Content creation	Draft captions, educational posts, reels scripts and case summaries from approved inputs.	Clinical accuracy and marketing review required.
FMD Main publishing	Prepare content for the clinic/dentist profile and submit it for approval before publishing to FMD Main.	Patient-case content must have valid consent and approval; never publish directly without required approval.
Patient communication	Draft responses, reminders, recall messages and follow-up scripts.	Respect channel consent, opt-outs, approved templates and escalation rules.
Clinic reporting	Prepare daily/weekly reports, payment sheets, recall sheets and operational summaries.	Show source data, date range and generated timestamp.
Inventory	Review stock, flag low items, draft purchase orders and compare recorded usage.	Purchases/orders require approval unless a tightly bounded policy is configured.
Staff coordination	Create tasks, prepare handovers, flag overdue actions and summarize workload.	Do not disclose restricted patient details to unauthorized staff.
Doctor personal assistance	Organize schedule, reminders, task lists, meeting notes and optional wellbeing check-ins.	Wellbeing feature is not diagnosis, treatment or emergency monitoring.
Dental knowledge support	Explain dental concepts, help draft patient education and surface references or clinic protocols.	Clinical decisions remain with the responsible dentist; cite sources where available.

7.3 Agent execution lifecycle

- **Receive:** Parse the instruction, identify user, clinic, intended outcome and relevant context.
- **Clarify:** Ask for missing details rather than guessing when the action could affect patients, finances, records or external communications.
- **Plan:** Break the request into bounded steps and identify tools/data needed.
- **Authorize:** Check user permissions, clinic policies, consent, action risk and approval requirements.
- **Preview:** Show affected records, recipients, content or changes for actions requiring confirmation.
- **Execute:** Call approved backend tools with idempotency, rate limits and appropriate safeguards.
- **Verify:** Confirm the actual system state; do not report success based only on an attempted API call.
- **Report:** Summarize completed, skipped, failed and pending steps with next actions.
- **Audit:** Store actor, request, tool/action, target, approvals, timestamps, result and relevant version identifiers.

7.4 Autonomy and approval matrix

Risk/action type	Default behavior
Read-only summaries and report preparation	May run automatically for authorized users.
Internal task creation and draft generation	May run automatically within configured scope; notify relevant user.
Routine reminders using approved templates	May run automatically only when consent, schedule, recipient and clinic policy are satisfied.
External messages not covered by an approved rule	Require preview/approval.
Appointment changes affecting patients	Require confirmation or operate only within explicit clinic-configured rules; log affected appointments.
Publishing content to FMD Main	Approval required. Patient-case content also requires documented patient consent and clinical/marketing review.
Editing signed/locked clinical records	Do not silently modify. Use an amendment workflow attributable to clinician.
Refunds, bank transfers, payment credentials or financial commitments	Require human authorization and any external banking/payment controls.
Clinical diagnosis, treatment selection or emergency decisions	AI must not autonomously decide or execute; route to clinician/professional care.

7.5 Tool orchestration and architecture requirements

- Use a model-orchestration layer that routes among language, vision, speech, retrieval and task-planning capabilities as needed.
- Expose business actions through typed, permission-checked tools/APIs—not unrestricted database access.
- Maintain explicit tool schemas, input validation, output validation, timeouts, retries and idempotency.
- Separate model reasoning from deterministic business rules such as appointment availability, permissions, prices and payment state.
- Use retrieval over approved clinic knowledge, templates and policies with access filtering.
- Support provider/model fallback and configurable cost/latency controls.
- Protect against prompt injection in uploaded files, messages, web content and retrieved material; treat external content as data, not instructions.
- Use task queues for long-running jobs and allow cancellation where safe.
- Provide a safe failure mode: no silent partial changes; show what happened and what remains.

7.6 Memory and context

- Separate short-lived task context, conversation context, clinic configuration and durable user preferences.
- Persist only useful, permitted information; provide controls to inspect/delete stored preferences where appropriate.
- Do not treat inferred clinical facts as verified medical records.
- Do not expose one clinic's memory, prompts, documents or tasks to another clinic.
- Allow the clinic to define retention rules for task histories and generated artifacts.

7.7 Cost, usage and observability

- Track model usage, task count, tool calls, latency, failures and estimated cost by tenant and feature.
- Support configurable quotas, alerts, model selection and cost ceilings.
- Monitor task completion rate and human correction/approval patterns.
- Do not optimize cost by bypassing safety checks or silently degrading clinically important outputs.

8. FMD Scholar standalone app

FMD Scholar is a separate app designed as a dentistry-focused, orchestrated AI workspace for students and academic users. It aims to support the full range of dental learning, viva preparation, presentations, research and academic work.

8.1 Access and pricing

Entitlement	Rule
Student access	A student who creates an FMD Main account and pays ■400/year receives FMD Scholar access included.
Included usage	Base access has defined usage limits. Exact quotas should be configurable and disclosed.
Optional upgrade	Tentative ■350 extra when included limits are exhausted. Billing period and exact benefit remain pending confirmation.
Eligibility	Scholar access is tied to valid student membership status; define grace period and renewal behavior.
Other user access	Dentists/professionals may have a separately defined entitlement if offered; do not assume inclusion without a product decision.

8.2 Academic assistant

- Clarify dental doubts across basic sciences, preclinical and clinical dentistry, subject to available validated knowledge and source quality.
- Explain concepts at selectable levels: beginner, undergraduate, postgraduate or specialist-oriented.
- Support follow-up questioning, quizzes, flashcards, revision plans and topic summaries.
- Generate viva questions, model answer outlines, examiner-style follow-ups and self-assessment sessions.
- Allow users to upload notes, lecture material or documents and ask questions grounded in those files, with source/page references where available.
- Clearly flag uncertainty, conflicting evidence and need to verify with textbooks, guidelines or faculty.

8.3 Presentation and seminar generation

- Generate presentation outlines and slide-by-slide content from a topic, audience, duration and desired depth.
- Support dental seminar, journal club, case presentation, classroom and conference formats.
- Allow users to set slide count, visual style, citation requirements and speaker notes.
- Create editable PowerPoint output when supported by the agreed file-generation service.
- Include references and citations that are traceable and not fabricated; flag missing source verification.
- Support user-provided papers and documents, with quotations and reuse limits handled appropriately.
- Allow review/edit/regenerate at slide, section or full-deck level.

8.4 Research and academic workflows

- Help formulate research questions, objectives, hypotheses and study outlines.
- Support literature-search planning, paper summaries, comparison matrices and evidence extraction from uploaded sources.
- Assist with methodology drafts, data dictionaries, survey/questionnaire drafts and statistical concept explanations.
- Support manuscript structure, abstract drafts, poster content and response-to-reviewer drafting.
- Do not fabricate studies, results, ethics approvals, references or data.
- Require user verification of references and prohibit presenting AI-generated content as completed research or original experimental findings.
- Include academic integrity reminders and disclose AI assistance where institutional policy requires it.

8.5 Scholar workspace

- Projects/workspaces for subjects, seminars, research projects and exam preparation.
- Saved conversations, uploaded documents, generated outputs and revision history.
- Searchable library with tags and user-controlled organization.
- Export, delete and retention controls.

- Usage meter showing included limits, remaining allowance and optional upgrade path.

9. FMD Admin and Accountant Portal

9.1 Admin overview

- Platform dashboard for user, clinic, subscription, booking, content, Concierge and operational metrics.
- User/provider verification queues and account status controls.
- Content reports, moderation queue, sponsored content and advertising management.
- Subscription catalog, entitlements, renewals, payment exceptions and support cases.
- Concierge enquiry oversight and assignment.
- System configuration, feature flags, service status, audit events and operational alerts.
- Role and access administration with privileged-action logging.

9.2 Accountant Portal — operating objective

The accountant should be able to handle routine company accounting and filing work independently through a dedicated portal, without routinely calling the founder for basic records, statements or transaction clarification. The portal should surface source documents, transaction details, deadlines and exceptions in a self-service manner.

Accounting capabilities

- Income ledger views for memberships, Clinic OS subscriptions, advertising/sponsored content and Concierge-related receipts.
- Expense recording and categorization, vendor bills, freelancer invoices, cloud/software bills, marketing expenses, payroll-related records and reimbursements.
- Bank statement import, transaction matching, reconciliation status and unresolved-item queue.
- Accounts payable/receivable tracking, due dates, payment status and supporting documents.
- Invoice, receipt, credit note and refund records, with references to original transactions.
- GST-related transaction data, input-tax-credit support records, tax-period summaries and exportable schedules.
- TDS and other tax-support records where applicable to the company's obligations.
- Filing calendar, document checklist, preparation status, submission evidence and due-date reminders.
- Trial balance, profit and loss, balance sheet, cash-flow summaries and configurable management reports.
- Monthly/quarterly/year-end close checklist, exception list and downloadable document bundle.
- Export/sync with a selected accounting platform such as TallyPrime or Zoho Books, subject to confirmed integration.

Independence and safeguards

- Give accountant a dedicated login, MFA and finance-only role.
- Show transaction source, payment reference, date, customer/vendor, tax fields and attached evidence to reduce follow-up questions.
- Provide an exceptions queue for missing invoice, unmatched bank entry, unclear category, failed payment or filing dependency.
- Allow accountant to prepare entries, reconciliations and filings workflow independently.
- Actual statutory filing must use valid professional/company authorization and approved external systems; portal should record filing status and evidence.
- Do not store bank passwords, OTPs or digital-signature secrets in FMD.
- Separate preparation from approval for high-risk payments, refunds, changes to bank/payment settings and other founder-controlled actions.
- Keep immutable audit history for edits, approvals, exports and filing-status changes.

The goal is accountant independence for routine work—not removing legally required company authorization, director responsibility, professional review or external portal access requirements.

9.3 Finance data separation

- Accountant sees company finance records and necessary transaction metadata, not clinical notes, patient images or unrelated private conversations.
- Clinic-level financial information is not automatically company-level FMD revenue.
- Where FMD facilitates clinic treatment payments, identify the merchant, settlement destination, platform fee and accounting treatment explicitly.
- Restrict exports by date range, legal entity, branch and permission.

10. Shared platform capabilities

Identity, profiles and organizations

- Common identity and verification services where practical.
- Organization/clinic membership and invitations.
- Entitlement service for membership, subscriptions and usage limits.
- Session management, device/session revocation and account recovery.

Search and discovery

- Search across providers, clinics, content and jobs with separate relevance rules.
- Location permission only when needed; allow manual location entry.
- Clearly label sponsored results and keep paid ranking distinguishable from organic matching.
- Provide filtering, sorting, pagination and no-result suggestions.

Media and documents

- Secure uploads with file type/size validation, malware scanning where available and access control.
- Image compression/transcoding and video processing as appropriate.
- Signed, expiring access links and lifecycle/retention rules.
- Document previews, download and deletion controls.

Payments and subscriptions

- Payment gateway abstraction, webhook verification and transaction ledger.
- Plan catalog, recurring billing where supported, renewals, grace periods and cancellation.
- Receipts, failed-payment recovery, refund workflow and reconciliation.
- Idempotent payment handling and clear settlement accounting.

Content and moderation

- Content creation, draft, review, publish, edit, archive and report lifecycle.
- Role-specific permissions and patient-consent checks for clinical case content.
- Moderation actions, appeals/support route and audit history.
- Advertising disclosure and sponsored placement controls.

Support

- In-app help, issue reporting, ticket status and support contact.
- Contextual help for failed bookings, payments, uploads and AI task execution.
- Internal assignment, escalation, response-time metrics and closure reason.

11. Technical architecture and integrations

The implementation should be modular, secure and maintainable. The following is a suggested reference architecture, not a mandate to use a particular vendor. The team should document any material deviation and its trade-offs.

11.1 Suggested stack

Layer	Suggested option / requirement
Mobile apps	Flutter or another cross-platform framework if it meets native UX, accessibility, voice, camera and release needs.
Web apps	React/Next.js or equivalent for FMD Admin, Accountant Portal and web-facing clinic workflows if required.
Backend	Modular monolith initially or service-oriented architecture where justified; typed APIs and clear domain boundaries.
Runtime	Node.js/NestJS or equivalent maintainable backend framework.
Primary database	PostgreSQL with tenant/organization scoping, migrations, backups and audit support.
Cache/queues	Redis and a durable job queue where appropriate for notifications, AI tasks and background work.
File storage	Private object storage such as S3-compatible storage with encryption and expiring access links.
AI orchestration	Provider-agnostic orchestration layer, model gateway, tool registry, retrieval and task execution services.
Infrastructure	Cloud hosting such as AWS or an equivalent provider, with dev/staging/production separation.
Observability	Centralized logs, metrics, tracing, error reporting, uptime monitoring and cost dashboards.

11.2 Logical architecture

Client apps (FMD Main, FMD Clinic, FMD Scholar, Admin/Accountant) → **API gateway/backend domain modules** → identity, profile, booking, content, records, clinic operations, billing, entitlements, Concierge, notifications and analytics. AI requests pass through a controlled **AI orchestration layer** and call approved tools via backend APIs. External services connect through adapters/webhooks.

11.3 Domain modules

- Identity and access management
- Profiles, verification and organizations
- Membership, subscription and entitlement
- Provider discovery and search
- Appointments and availability
- Patient records and document storage
- Content, social graph and moderation
- Concierge enquiries and coordination
- Clinic workflow, billing and inventory
- AI conversation, orchestration, tools, task queue and audit
- Scholar workspace, documents and generation jobs
- Finance/accounting exports and reconciliation
- Notifications and communications
- Analytics, support and admin configuration

11.4 API and integration expectations

- Version APIs and document request/response schemas, auth, permissions, errors and rate limits.
- Use webhooks for payment status and external events; verify signatures and prevent replay.
- Use idempotency keys for payment, booking and task actions where duplicates would be harmful.
- Maintain integration adapters so payment, messaging, speech, maps, LLM and accounting vendors can be replaced.
- Provide sandbox credentials and integration test environments.
- Do not place secret keys in mobile clients or source repositories.

11.5 External services to evaluate

Service area	Possible integration / notes
AI models	OpenAI, Gemini, Claude or other suitable providers; compare quality, tool calling, multimodal support, data terms and cost.
Speech	Speech-to-text and text-to-speech for voice; evaluate Indian-language performance and consent requirements.
Payments	Razorpay, PayU, PhonePe or other suitable gateway; determine recurring billing, refunds, settlements and webhook support.
Messaging	SMS, WhatsApp Business, email and push providers subject to consent, templates and policy requirements.
Maps/location	Geocoding, maps and distance estimates with user permission and provider terms.
Accounting	TallyPrime or Zoho Books; confirm API capability, sync direction and accountant workflow.
Cloud	AWS or equivalent for compute, storage, database, backups, monitoring and security.
File generation	PowerPoint/PDF/document generation for Scholar and exports.
Analytics	Privacy-conscious product analytics and business reporting.

11.6 Environments and release management

- Separate development, staging and production environments.
- Automated build/test/deploy pipeline with review and rollback procedures.
- Configuration and secrets managed outside source code.
- Database migration plan and backward-compatible API changes where practical.
- App-store account ownership, signing assets and deployment access transferred to FMD.

12. Core data entities

The engineering team should produce a detailed ERD and data dictionary. At minimum, plan for the following logical entities:

Domain	Core entities / relationships
Identity	User, credential, session, role, permission, consent, organization membership.
Membership	Plan, subscription, entitlement, usage counter, renewal, invoice, payment transaction.
Provider directory	Dentist profile, clinic profile, qualification/verification record, service, location, availability.
Booking	Appointment, slot, resource, booking request, status event, cancellation/reschedule event.
Patient records	Patient profile, clinical document, treatment episode, treatment plan, clinical note, sharing grant, access event.

Domain	Core entities / relationships
Content	Post, media asset, comment, reaction, follow, report, moderation action, campaign/sponsored placement.
Concierge	Enquiry, assignment, contact attempt, coordination note, consent, status event, service outcome.
Clinic operations	Clinic workspace, branch, staff, task, inventory item, stock movement, supplier, purchase order.
Clinic finance	Estimate, invoice, receipt, payment, refund, adjustment, ledger reference.
AI	Conversation, message, task, plan step, tool invocation, approval, result, artifact, model usage, audit event.
Scholar	Workspace, project, document, source, generated deck, quiz, study plan, usage event.
Accounting	Company entity, ledger entry, bank import, reconciliation, vendor, expense, filing period, filing artifact, close checklist.
Platform ops	Support ticket, notification, feature flag, audit event, admin action.

Sensitive health data should be logically and technically separated from general profile, content and marketing data. Define ownership, retention, deletion and export behavior for each entity.

13. Security, privacy, clinical safety and compliance

Security baseline

- TLS for data in transit and encryption at rest for databases, backups and object storage.
- Strong password/OTP controls, MFA for privileged roles, session revocation and brute-force protection.
- Role- and organization-scoped authorization enforced server-side on every protected request.
- Audit logs for access to sensitive records, privileged changes, finance actions and AI-executed tasks.
- Secure secret management, dependency scanning, vulnerability remediation and incident response.
- Backups with tested restore procedures; define recovery point and recovery time objectives with the team.
- Rate limits, abuse controls, input validation, output encoding and protection against common web/API vulnerabilities.

Privacy and consent

- Provide understandable privacy notices and purpose-specific consent where needed.
- Collect minimum necessary data and define lawful purpose, access, retention and deletion rules.
- Support user access/export and deletion requests subject to applicable legal retention duties.
- Record patient consent for sharing records and publishing identifiable case material.
- Do not use patient records to train external models unless separately authorized and legally reviewed.
- Review data processing terms, cross-border transfers, vendor retention and model-provider settings before launch.

Clinical and AI safety

- FMD Main AI is educational and navigational, not a diagnostic substitute.
- FMD Clinic AI may prepare drafts and execute administrative actions, but clinical judgment remains with the responsible dentist.
- Clinical content and escalation language should be reviewed by qualified dental professionals.
- Show source/limitations for AI-generated academic and clinical summaries where possible.
- Prevent unsupported prescriptions, treatment guarantees and fabricated citations.
- Require human review for clinical notes, patient-case publication and high-impact external communications.
- Use explicit risk tiers, approvals and emergency escalation language; do not imply AI is monitoring emergencies.

India-specific legal and regulatory review

Before launch, obtain qualified Indian legal, tax, privacy and professional-regulatory review for the actual operating model, including applicable data-protection requirements, telemedicine/clinical boundaries if relevant, professional advertising and ethics rules, consumer protection, GST/TDS, payment settlement and electronic records. This PDR identifies design controls; it is not a compliance certification.

AI threat and failure controls

- Treat user uploads, retrieved documents and external messages as untrusted data.
- Do not allow a prompt or document to expand permissions or override system safety rules.
- Require confirmation for irreversible or high-impact actions.
- Detect duplicate execution and partial failures.
- Provide human override, cancellation and incident review.
- Retain minimal audit evidence needed to reconstruct what happened.

14. Commercial model and billing

Audience/product	Current stated price	Entitlement / notes
FMD Main — patient	Free	Public patient access; any paid services disclosed separately.
FMD Main — dentist/clinic account	■550/year	Professional account on public app; separate from Clinic OS.
FMD Main — student	■400/year	Student membership includes FMD Scholar access within base usage limits.
FMD Main — dental company/corporate	■2,000/year	Corporate account; define included profile and promotional entitlements.
FMD Clinic OS	■2,000/month	Clinic-management subscription and agentic AI access; define fair-use/AI limits and any add-ons.
FMD Scholar optional upgrade	Tentatively ■350 extra	Triggered when included usage limits are exhausted; billing period and exact upgrade benefit pending confirmation.
FMD Concierge	Not finalized	Enquiry-led service; quote/terms communicated during personal follow-up.

Billing requirements

- Display price, tax treatment, renewal frequency, included features, usage limits and cancellation/refund terms before purchase.
- Keep membership and Clinic OS subscriptions distinct in catalog and receipts.
- Provide admin-configurable plans, coupons/discounts if enabled, grace periods and renewal notices.
- Record payment attempts, gateway references, settlement status, refunds and invoice links.
- Use verified gateway webhooks as the source of payment-state updates; reconcile exceptions.
- Do not count clinic treatment fees as FMD revenue unless the contractual and accounting model establishes that treatment fee belongs to FMD.
- Show Scholar usage and upgrade path before users hit limits; avoid unexpected hard stops or hidden charges.

15. Notifications and communications

Event	Potential channel	Rules
Appointment confirmation/reminder	Push, SMS, WhatsApp or email	Consent, clinic settings, quiet hours, template and opt-out.
Booking change/cancellation	Push plus configured fallback	Show accurate status and next action.
Membership renewal/payment failure	Push/email/SMS	Include amount, due date and secure payment route.
Concierge enquiry received/status	In-app/push/email	Do not promise a confirmed service until confirmed by staff.
Clinic AI task complete/approval needed	In-app/push/email	Include task status, approval link and failure details.
Recall follow-up	Clinic-configured channel	Only use clinic-approved recall rules and valid communication permissions.
Security/account event	Email/push/in-app	Do not expose sensitive information in lock-screen text.

Maintain notification templates, localization, delivery logs, retries and user/clinic preference controls. Avoid sending sensitive clinical details in notification previews.

16. Analytics and reporting

FMD Main

- Registration and membership conversion by user type.
- AI usage, helpfulness feedback, unresolved query rate and escalation patterns.
- Search-to-profile and profile-to-booking funnel.
- Content discovery, saves, engagement, reports and sponsored-content disclosure metrics.
- Concierge enquiry volume, response time and outcome.

FMD Clinic

- Active clinics, staff adoption and task completion rates.
- AI tasks started, completed, blocked, failed, approved and corrected.
- Time-to-completion and human review rate by task type.
- Appointment utilization, recall completion, no-shows, collections and outstanding balances.
- AI model cost, tool errors, latency and usage by clinic.

FMD Scholar

- Student entitlement activation, active usage and limit consumption.
- Feature use across doubt clarification, viva, PPT and research workflows.
- Generation success, citation verification flags, export success and upgrade conversion.

Admin and accounting

- Subscription revenue, refunds, failed payments, expenses and reconciliation status.
- Filing deadlines, unresolved finance exceptions and close completion.
- Support backlog, verification queue and moderation response times.

Define metric formulas, event names, owner, retention, privacy controls and dashboard access. Avoid exposing identifiable health information in general product analytics.

17. Non-functional requirements

Area	Requirement
Performance	Set measurable targets for common app screens, search, booking, document upload and AI response/task status. Agree targets after baseline and hosting design.
Availability	Define service-level objectives for core services and planned maintenance communication.
Scalability	Support growth in users, clinics, content, files and concurrent AI jobs through measured capacity planning.
Reliability	Idempotent critical actions, durable queues, retries with limits, dead-letter handling and recovery procedures.
Accessibility	Readable typography, screen-reader labels, sufficient contrast, keyboard support for web and accessible controls.
Localization	Externalize strings, support Indian date/time/currency formats and test multilingual rendering.
Compatibility	Define supported iOS/Android versions, browser matrix and device testing targets.
Maintainability	Modular code, code review, linting, tests, API documentation and architecture decision records.
Observability	Central logs, metrics, tracing, alerts, AI-task traceability and cost monitoring.
Portability	Avoid unnecessary vendor lock-in; document export paths and replacement options for critical services.

18. End-to-end user journeys

Patient: concern to appointment

Patient opens FMD Main → asks the AI about a dental concern → receives educational guidance and appropriate caveats → requests provider suggestions → reviews dentists/clinics and availability → selects a slot → confirms booking in the booking flow → receives status and reminders → optionally stores related records.

Patient: Concierge request

Patient opens Concierge → reviews service scope → submits enquiry and contact preference → receives reference/status → FMD coordinator contacts patient → clarifies requirements → coordinates provider/service options → patient confirms any proposed arrangement → enquiry is closed with outcome.

Clinic: AI prepares recall work

Authorized staff asks AI for patients due for recall → AI applies clinic-configured criteria → previews the list and date range → authorized user approves any outreach batch → messages are sent through configured channels → delivery/outcome statuses are recorded → AI produces a follow-up summary.

Clinic: create and publish a case post

Dentist selects an eligible case and provides approved media/context → system checks consent record and required fields → AI drafts a de-identified summary/caption and content plan → dentist reviews clinical accuracy and privacy → authorized user approves → post is submitted/published to FMD Main → action and consent reference are logged.

Student: academic workflow

Student pays ■400/year for FMD Main student membership → activates FMD Scholar → selects subject/topic and desired depth → asks doubts or starts viva practice → uploads sources if needed → generates seminar outline/deck → reviews references and content → exports and saves work → sees usage meter and optional upgrade when limits are reached.

Accountant: month close

Accountant signs in with MFA → reviews imported transactions and source documents → categorizes expenses/income → reconciles bank entries → resolves exceptions or requests missing documentation through the portal → prepares tax-period schedules and reports → completes filing workflow in authorized external systems → uploads evidence/status → closes period and exports reports.

FMD Main AI boundary

The public assistant can guide and prepare users for actions, but it does not independently create bookings, send messages, submit forms or make payments. The user remains in control of the action.

19. Acceptance criteria and QA

Each feature should have testable acceptance criteria in the implementation backlog. The following baseline applies across products:

- Authentication, account recovery, role checks and organization scoping are tested for allowed and denied cases.
- Critical workflows have happy-path, validation, cancellation, timeout, duplicate request and partial-failure tests.
- Payment and booking state changes are verified against backend/gateway records rather than client-only state.
- AI task execution checks permission before tool use, requests required approvals, verifies outcomes and writes audit records.
- AI must not claim an action succeeded when it failed, timed out or remains unverified.
- Patient-case publishing cannot proceed without required consent and authorized approval.
- Public AI cannot autonomously perform patient tasks such as booking or contacting providers.
- Clinic AI cannot bypass locked clinical records, finance approvals or clinic tool permissions.
- Scholar-generated references are traceable or explicitly flagged for verification; fabricated sources are treated as defects.
- Accountant exports respect legal entity, date range, role and data-scope restrictions.
- Accessibility, localization, app-store flows, notification opt-outs and deletion/export routes are tested.
- Security testing includes tenant isolation, access control, upload validation, secrets handling and prompt-injection scenarios.

AI evaluation

- Create a dentistry-reviewed test set for common educational questions, urgent-symptom language, ambiguous prompts and unsafe requests.
- Measure factuality, calibration, citation quality where applicable, tool selection, task success, refusal/escalation behavior and cost.
- Test multilingual quality separately for each supported language; do not assume English quality transfers.
- Run red-team tests for cross-tenant leakage, unauthorized actions, malicious documents and approval bypass.
- Keep human review in the loop for clinically consequential content and high-impact tasks.

20. Delivery and handover

Required developer deliverables

- Approved requirements traceability matrix linking PDR features to screens, APIs, data entities and tests.
- UX flows, wireframes/design system and responsive layouts for all agreed surfaces.
- Architecture diagram, deployment diagram, ERD, data dictionary and integration inventory.
- API documentation, webhook contracts, role/permission matrix and AI tool registry.
- Source code repositories owned or controlled by FMD, with documented branch/review practices.
- Development, staging and production environments with deployment and rollback instructions.
- Automated tests, QA results, known defects and security review findings.
- App-store builds and release assets, with ownership/access under FMD.

Delivery and handover

• Admin guides, accountant guide, clinic onboarding guide, AI task/approval guide and support runbook.

• Credentials and secrets handover through a secure method; no secrets embedded in documents or source.

• Backup/restore procedure, monitoring, incident response and maintenance plan.

• Third-party vendor list, subscription costs, API limits, data-processing terms and renewal owners.

Scope management

This PDR records the full product vision. The delivery team must provide a transparent estimate, dependencies, assumptions, exclusions, recurring infrastructure/API costs and a change-control process. Any reduction or sequencing must be agreed explicitly by FMD; it must not silently redefine the product vision.

Definition of done

• Feature meets approved acceptance criteria and has passed relevant tests.

• Permission, privacy, audit and failure behavior are implemented—not deferred as documentation only.

• UI includes loading, empty, error, success and recovery states.

• Telemetry and support diagnostics are available without unnecessary sensitive data.

• Documentation and ownership/handover items are complete.

• FMD product owner accepts the feature against the agreed scope.

21. Open decisions and assumptions

Item	Current understanding	Action needed
Scholar upgrade	Tentative extra tier after included usage limits are exhausted.	Confirm billing period and whether this buys more usage, persistent memory, or both.
Scholar quotas	Base access included with student membership.	Define limits by feature/model, reset period, overage behavior and transparency.
Clinic AI limits	Agentic AI is the main Clinic OS USP.	Define fair-use allowance, included channels, model costs and paid add-ons if any.
Clinic AI autonomy	Runs individual tasks; case content publishing requires approval; high-impact actions have safeguards.	Approve detailed action-by-action permission matrix and default settings.
Concierge commercial terms	Enquiry and personal coordination model.	Define service areas, fees, response expectations, cancellation and liability terms.
Accounting integration	Accountant should independently handle accounting and filings workflow.	Select TallyPrime/Zoho Books/other, integration method and filing portals.
Statutory filing authority	Portal supports preparation, records and evidence; actual filing requires valid authorization.	Confirm company roles, CA engagement, digital-signature and portal access requirements.
Public app booking/payment model	FMD Main can guide and support booking; actual payment flow not fully specified.	Confirm booking fees, payment collection, cancellation/refund and settlement model.
Membership verification	Professional, student and corporate memberships have different prices/benefits.	Define verification documents, renewal/grace rules and account-type changes.
Content import from social platforms	Content creator import has been discussed previously.	Confirm permitted platform APIs, rights, attribution and manual alternatives.
Doctor wellbeing checks	Requested as a personal-assistant capability in Clinic AI.	Define whether this means schedule/break prompts, self-reported check-ins or another narrowly scoped feature.
Compliance review	India-specific privacy, tax, professional and consumer obligations apply depending on implementation.	Obtain qualified legal, clinical, tax and security review before launch.

Assumptions in this document are intentionally visible. Product decisions that materially affect legal obligations, data flows or user trust should be resolved before the corresponding feature is built.

FMD | FINDMYDENTIST

Master PDR — Expanded Product & Technical Annex

Supplement to the existing FMD Master Product Requirements Document
Prepared 4 October 2026

Purpose. This annex adds detailed requirements for the requested product areas and expands the operational, AI, onboarding, measurement and accessibility requirements. It is intended to be read together with the original PDR. It does not silently change existing prices, commitments, or the original product vision.

Scope note. The original PDR describes the full intended ecosystem rather than a committed V1 scope. The features below are product requirements for planning and estimation; launch phase, integrations, service levels, quotas and final acceptance thresholds must be approved in the delivery scope.

A. Digital treatment warranty certificates

FMD should support a digital treatment warranty certificate where a participating clinic offers a warranty or guarantee for an eligible treatment. The certificate is an organized record of the clinic's stated terms, not an independent promise by FMD that treatment outcomes are guaranteed.

A1. Certificate creation and lifecycle

• Create a certificate from a completed treatment/eligible invoice or an authorized clinic workflow; support manual issuance with required fields.

• Include unique certificate ID, patient and clinic references, treating provider, treatment/procedure category, relevant tooth/site where appropriate, treatment date, issue date, validity period, covered items, exclusions, conditions, follow-up requirements, and issuing clinic contact.

• Support clinic-defined templates and eligibility rules, with versioning and a record of the terms accepted/issued at the time.

• Deliver the certificate to the patient's FMD record and optionally by secure link/PDF; provide a verification view using a non-guessable identifier or QR code without exposing unnecessary patient data.

• Support active, expired, amended, voided, disputed and transferred states where the clinic's policy permits; preserve an immutable history of material changes.

• Allow the clinic to attach supporting documents and record follow-up/maintenance visits relevant to the certificate.

• Provide patient-facing reminders before expiry or required review dates only where consent and communication preferences allow.

A2. Rules and safeguards

• Clinic is the issuer and owner of warranty terms; FMD must not invent, broaden or interpret coverage.

• Display terms in clear language and make exclusions, conditions, validity and responsible clinic prominent.

• Corrections should create a new version or amendment record rather than silently overwrite issued terms.

• Provide dispute/contact routing and an audit trail of issuance, viewing, amendment and revocation.

• Acceptance: a certificate can be issued, securely viewed, verified, amended and expired; access control and patient export/deletion rules are tested.

B. Unified dental treatment journey

FMD should connect discovery, consultation, treatment planning, treatment delivery and follow-up into a patient-visible journey, with clinic-controlled records and permissions. It should reduce fragmentation without replacing the clinical record or dentist's judgment.

Journey stage	Capabilities	Key controls
Discover and triage	Patient describes concern; public assistant provides general guidance and specialty suggestions; clinic/dentist discovery.	No definitive diagnosis; urgent concerns use reviewed escalation language.
Appointment and consultation	Request/book appointment, reminders, visit details, consultation notes or uploaded documents where supported.	Availability and booking status validated by clinic system; consent and communication preferences.
Estimate and treatment plan	Clinic shares estimate, proposed stages, expected visits and patient-facing explanation.	Dentist/clinic authors and approves; version and acknowledgement history.
Consent and decisions	Capture acknowledgement or link to clinic consent workflow; record questions and decision status.	Consent is specific and informed; do not treat app acceptance as a substitute for legally/clinically required consent.
Treatment and records	Timeline of completed visits, documents, images, invoices and relevant instructions.	Role-based access; clinical record remains authoritative; avoid duplicate or conflicting records.
Payment and warranty	Show invoice/payment status from authoritative integration and eligible digital warranty certificate.	No fabricated payment state; clinic terms govern warranties/refunds.
Recall and follow-up	Clinic-configured follow-up reminders, patient check-ins and next-visit scheduling.	Opt-out, consent, approved templates and human escalation for clinical replies.
Closure and continuity	Patient can view/export permitted journey history and share selected records with a new provider.	Granular sharing, expiry/revocation, access logs and data minimization.

B1. Journey data and experience

• Show a chronological timeline with clear status labels such as planned, scheduled, completed, pending patient action and cancelled.

• Support multiple treatment plans and episodes per patient without mixing unrelated cases.

• Allow clinic staff to correct administrative timeline errors with attribution and history.

• Show patients only information released by the clinic and permitted by the patient's access rights.

• Acceptance: test a full journey from discovery/booking through completed treatment, invoice, certificate (if applicable), follow-up and export.

C. Content import and creator workflows

FMD's content hub should make it practical for dental creators and clinics to bring content into FMD while respecting platform terms, copyright, attribution, patient consent and content moderation.

• Support permitted imports through official APIs, authorized sharing/embedding, user-uploaded files or manual link entry. Do not scrape or bypass platform restrictions.

• Capture source URL/platform, creator/rights holder, title/caption, media type, attribution, import date, license/permission status and moderation state.

• Where direct media import is not permitted, use an attributed link or embed if allowed, or prompt the user to upload content they own or have permission to use.

• Support draft review, crop/preview, captions, tags, specialty/topic labels, accessibility text and scheduled publishing where supported.

• Provide duplicate detection, broken-link handling, content ownership declarations, takedown/reporting workflow and audit history.

• Require consent and de-identification review for clinical cases, patient images, testimonials and before/after content.

• Separate imported source material from FMD-hosted copies; track whether FMD has rights to store, transform, recommend or monetize the content.

C1. Moderation and acceptance

• Create report categories for misinformation, misleading claims, privacy/consent, harassment, spam, copyright and inappropriate content.

• Support review queues, decisions, appeal/escalation and repeat-offender controls.

• Acceptance: test permitted import paths, attribution display, unsupported-platform fallback, consent gates, takedown and moderation.

D. AI personalisation

Personalisation should make FMD more relevant to patients, dentists, students and clinics while remaining transparent, controllable and privacy-conscious.

Audience	Personalisation examples	Controls
Patients	Relevant dental education, saved providers, reminders, journey prompts and content based on declared interests or permitted activity.	Explain why content is suggested; allow preference editing, reset and opt-out; do not infer sensitive diagnoses for advertising.
Dentists	Specialty-relevant content, professional updates, workflow shortcuts and clinic insights.	Role and specialty settings are user-editable; avoid undisclosed paid ranking.
Students / Scholar users	Topic-based learning, revision plans, preferred learning format and progress-aware suggestions.	Separate learning personalization from patient/clinic data; disclose AI-generated material and source limitations.
Clinic teams	Role-specific dashboards, recurring task suggestions and operational summaries.	Use authorized clinic data only; staff access follows clinic roles.

• Use explicit preferences first; use behavioral signals only with appropriate notice, lawful basis and controls.

• Provide controls to inspect, correct, reset or disable personalization and recommendations.

• Keep paid promotion distinguishable from organic recommendations; define ranking factors and avoid misleading clinical endorsements.

• Do not use patient records to target ads or train external models absent explicit authorization and legal review.

• Evaluate recommendation relevance, diversity, freshness, unwanted repetition, fairness and user controls.

• Acceptance: users can change preferences, reset personalization, opt out where offered and understand sponsored placement.

E. Clinic onboarding and activation

Clinic onboarding must be a guided, measurable process from lead or invitation through verified setup, staff training and first successful workflow.

Step	Requirements
Lead and qualification	Capture clinic name, locations, contact, clinic type, size band, source, consent to contact and onboarding owner.

Step	Requirements
Identity and verification	Verify clinic/provider details using an approved checklist; collect only necessary documents and restrict access.
Organization setup	Create tenant and branches; configure time zone, operating hours, services, chair/provider availability and booking rules.
Team and roles	Invite owners, dentists, front desk and finance roles; explain permissions; require secure account setup.
Commercial setup	Record selected plan, trial/contract terms, billing contact, invoice details and accepted terms; keep Clinic OS subscription distinct from any directory membership.
Data and integrations	Choose manual setup, template import, supported integration or migration; map ownership and permissions.
Training and launch	Provide setup checklist, demo data, role-specific guides, training and go-live confirmation.
Activation and follow-up	Track first appointment, first patient record, first invoice/other agreed activation events; schedule check-in and capture issues.

• Provide an onboarding dashboard with owner, status, blockers, missing fields, due dates and completion evidence.

• Support assisted onboarding for clinics that need help and self-service onboarding for simpler setups.

• Provide data-processing and access explanations before uploading real patient data.

• Define handoff from sales/onboarding to customer support and account management.

• Acceptance: a new clinic can be created, verified, configured, trained and activated with a complete audit trail.

F. Data migration and interoperability

Migration must be treated as a controlled project, not an unreviewed bulk upload. Scope depends on the clinic's existing software, data quality, export formats and available integrations.

• Inventory source systems and data categories: patients, appointments, providers, services, clinical notes, attachments, invoices, balances, consent records and relevant history.

• Agree the migration scope, legal authority, clinic instructions, retention, transfer method, downtime window and rollback plan before transfer.

• Provide supported CSV/template imports first; evaluate specific PMS integrations only after access, vendor terms and technical feasibility are confirmed.

• Map fields and identifiers; normalize dates, phone formats, codes and duplicate patient records without losing source references.

• Run pre-import validation, sample migration, error report, clinic review and explicit sign-off before production import.

• Preserve provenance and timestamps where available; flag missing, ambiguous, malformed or conflicting records rather than silently guessing.

• Encrypt transfer and staging data, restrict access, log actions, delete temporary copies under the agreed retention plan and document completion.

• Provide reconciliation counts by entity, rejected-row reports, attachment checks and a rollback/recovery procedure.

F1. Migration acceptance checklist

• Source-to-target field mapping approved by clinic.

• Sample records reviewed and signed off.

• Counts and key totals reconciled; exceptions documented.

• Duplicate handling and attachment access tested.

• Permissions, tenant isolation, export and deletion behavior tested.

• Cutover, rollback, support contact and post-migration validation documented.

G. Patient feedback, reviews and experience recovery

FMD should capture structured patient feedback at appropriate points in the treatment journey and provide clinics with a responsible way to respond and improve.

• Support configurable feedback invitations after appointments, treatment milestones or journey closure; honor consent, contact preferences and frequency limits.

• Collect private service feedback separately from public reviews. Make it clear whether a response is private to the clinic or publicly visible.

• Allow ratings and structured topics such as appointment access, communication, environment and overall experience; avoid soliciting clinical outcome claims as simple popularity scores.

• Provide clinic response tools, internal tagging, trend summaries and escalation for safety, privacy, discrimination or serious complaints.

• Use moderation rules for abusive content, personal data, conflicts of interest, spam, fabricated reviews and incentives.

• Disclose verified-visit rules and any incentives; do not suppress negative feedback or sell review outcomes.

• Provide report, correction, appeal and takedown workflows with documented reasons.

• Acceptance: test invitation consent/frequency, private/public distinction, clinic response, moderation, report and escalation.

H. Clinic customer support and service operations

Clinic support is a product capability and an operating function. The product should let clinics raise, track and resolve issues without exposing unnecessary patient information.

• Provide in-app help center, searchable guides, onboarding materials, support request form and ticket history.

• Classify issues by product area, severity, business impact, affected clinic/branch, version and reproducibility.

• Allow secure attachment of logs/screenshots with warnings and redaction guidance; discourage sharing patient-identifiable data unless necessary and authorized.

• Define support hours, response targets, escalation owners and service levels before publishing them. Do not promise 24/7 support unless staffing and operations support it.

• Provide status updates, workaround, resolution summary, closure confirmation and reopen path.

• Create internal playbooks for booking/payment incidents, access problems, data migration, AI safety incidents, suspected breaches and vendor outages.

• Track ticket volume, first response, resolution time, reopen rate, satisfaction and recurring root causes.

• Acceptance: clinic can create and track a ticket; staff can triage/escalate; sensitive details are access-controlled and auditable.

I. Referral and loyalty system

A referral and loyalty layer may support patient growth and repeat engagement, but must be designed around applicable dental-professional rules and must not create prohibited inducements, fee splitting or improper clinical referrals.

• Separate patient-to-platform invitations from dentist-to-patient or provider-to-provider clinical referrals.

• Use referral codes or links with attribution, campaign/source tracking, fraud checks, expiry, eligibility and transparent terms.

• Support non-clinical engagement rewards or platform benefits only after legal review; do not reward dentists for directing clinical treatment or paying for patient referrals.

• Allow clinics to define permitted patient engagement programs subject to FMD policy and applicable law; clearly identify who funds and fulfills any benefit.

• Track issued, pending, qualified, redeemed, expired, reversed and disputed rewards with an audit trail.

• Prevent self-referrals, duplicate accounts, fabricated appointments, chargeback abuse and manipulation.

• Disclose material conditions and avoid dark patterns or pressure to undergo treatment.

• Acceptance: referral attribution and reward ledger reconcile; eligibility, expiry, abuse and reversal cases are tested.

Open legal decision: Any reward, discount, cashback, referral benefit or clinic-funded incentive requires India-specific legal and professional-ethics review before activation. Keep the feature configurable and disabled until approved.

J. Business intelligence and analytics

FMD should provide decision-useful analytics for clinics and platform operators while separating operational reporting from clinical decision-making and marketing use.

Analytics area	Example metrics / views	Guardrails
Clinic operations	Appointments by status, utilization, cancellations, no-shows, pending tasks, service mix.	Define numerator/denominator and time zone; show source freshness.
Patient journey	Booking-to-visit conversion, plan stage status, follow-up completion, certificate issuance where applicable.	Use minimum aggregation thresholds; restrict row-level detail by role.
Financial operations	Invoice totals, collections, outstanding balances, refunds and reconciliation status where integrated.	Do not label estimates as actuals; reconcile with source system and accounting provider.

Analytics area	Example metrics / views	Guardrails
Growth and retention	Clinic activation, active clinics, cohort retention, referral source, patient engagement.	Consent and lawful use; distinguish organic from paid acquisition.
AI operations	Tasks, verified completion, failures, approval rates, cost per task, latency and user corrections.	Avoid exposing sensitive prompt/record contents in dashboards.
Platform health	Uptime, error rate, queue delay, integration health, support volume and incidents.	Role-based access; incident data protected.

• Provide date range, branch, provider and service filters according to role and data access.

• Define metric names, formulas, exclusions, refresh cadence and source of truth in a metric dictionary.

• Support CSV/PDF export with permissions, watermarking or audit where appropriate.

• Use aggregated or de-identified views for platform-wide insights; prevent clinic-to-clinic data leakage.

• Do not present analytics as clinical outcome evidence unless the methodology, data quality and required review support that claim.

• Acceptance: reconcile sample dashboards against source records and verify tenant/role access.

K. Accessibility and inclusive design

Accessibility and inclusive design apply across FMD Main, Clinic OS, Scholar, Concierge flows, support and administrative interfaces—not only to a separate accessibility screen.

• Design for screen readers, keyboard navigation, visible focus, logical headings, labels, error announcements and meaningful control names.

• Maintain sufficient color contrast; never communicate status by color alone; support text resizing and responsive layouts.

• Use plain language, clear instructions, consistent navigation and helpful validation messages.

• Provide captions/transcripts for relevant audio/video content and text alternatives for meaningful images and charts.

• Design touch targets and forms for mobile use, low bandwidth and users with varied digital literacy.

• Support localization architecture for Indian languages; ensure translations, clinical terms and consent copy are reviewed rather than machine-published unchecked.

• Consider users with visual, hearing, motor, cognitive and temporary impairments, older adults and caregivers.

• Offer alternatives when a feature cannot be completed accessibly, including human assistance or non-voice workflows.

• Test with automated accessibility checks and representative users; target WCAG 2.2 AA as the design benchmark, subject to specialist validation.

Accessibility acceptance should include keyboard-only completion of core journeys, screen-reader testing on supported devices, contrast and text scaling checks, accessible authentication, form error recovery and review of localized content.

L. Cross-feature data, consent and governance

• Use a consistent patient/clinic identity model while keeping permissions and purposes separate across public discovery, clinical records, feedback, content, marketing, loyalty and analytics.

• Record the purpose and permission basis for each data flow; do not assume that consent for treatment or appointment messages also permits marketing, personalization or public content.

• Provide a data-flow inventory covering mobile apps, backend, AI providers, messaging, payment, analytics, support tools, backups and migration staging.

• Maintain a product decision log for unresolved choices, an owner, decision date and impact on scope/cost.

• Require clinical review for patient-facing dental guidance, urgent escalation, treatment explanations and case-content workflows.

• Require security/privacy review for new integrations, AI data flows, public sharing, rewards, migrations and analytics exports.

• Define release gates, incident severity, escalation contacts, rollback authority and vendor exit/data portability.

M. Cross-feature end-to-end acceptance scenarios

Scenario	Expected result
Treatment with certificate	Clinic completes eligible treatment, issues certificate with correct terms, patient views it securely, later verifies or exports it.
Journey with changed appointment	Patient receives permitted update; journey timeline and clinic schedule agree; audit shows who changed it.

M. Cross-feature end-to-end acceptance scenarios (continued)

Scenario	Expected result
Clinic onboarding with migration	Clinic is verified, roles set, sample data reviewed, migration reconciled and launch checklist completed.
Negative patient feedback	Feedback reaches correct private/public path, clinic can respond appropriately, serious complaint is escalated and audited.
Support incident	Clinic submits issue, support tracks status, access to attachments is restricted, resolution is documented.
Referral abuse attempt	Duplicate/self-referral is detected or flagged; reward is held/reversed according to transparent rules.
BI tenant isolation	A clinic user cannot see another clinic's patients, metrics or exports; aggregate platform reports meet privacy rules.
Accessibility core journey	A user completes discovery/booking or a clinic task with keyboard and screen reader and can recover from validation errors.

N. Decisions to finalize before development commitments

• Which features are in the first release versus later releases, and what is the explicit out-of-scope list?

• Warranty: eligible treatments/clinics, who authors terms, certificate validity and dispute handling.

• Treatment journey: authoritative record source, patient-visible stages and clinic workflow.

• Content import: supported platforms and permitted import modes after API, terms and rights review.

• Personalization: allowed signals, opt-outs, sponsored ranking rules and retention.

• Onboarding/migration: initial supported formats, assisted onboarding responsibility and supported legacy systems.

• Feedback: public reviews at launch, verification method, moderation owner and appeal process.

• Support: hours, channels, severity levels, response targets and staffing.

• Referral/loyalty: legally approved benefit types, funding, redemption, fraud and accounting treatment.

• BI: launch dashboards, metric definitions, exports and role permissions.

• Accessibility: supported languages/devices, test plan, target standard and remediation ownership.

• Commercial and technical dependencies: payment/accounting providers, messaging providers, hosting/model vendors and data-processing terms.

Implementation recommendation. Treat this annex as requirements input, then create a separate release-specific SRS/Statement of Work with feature IDs, screen/API details, dependencies, test cases, owners, estimates and acceptance sign-off.